AEGIS

CMS-0042-NC

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About the Digital Consortium and Aegis Ventures

This response to CMS-0042-NC reflects the perspectives of Aegis Ventures and the Digital Consortium—an alliance of 11 leading, not-for-profit health systems across the United States—guided by the leadership of Dr. John Noseworthy, President and CEO Emeritus of Mayo Clinic.

At Aegis Ventures, we build companies that solve structural problems in healthcare. Our work focuses on care navigation, administrative simplification, and digital tools that work for patients and their caregivers. Through the Consortium, we co-develop solutions with health systems and patients, focusing on what works in real-world care delivery. We've built products that help patients manage chronic conditions, reduce unnecessary visits, and improve follow-through after hospital discharge—while addressing the operational barriers that slow adoption.

This gives us a close view into the gaps that patients, caregivers, and providers face every day. We understand the technical and policy infrastructure needed to scale digital tools across Medicare populations. And we understand what it takes to earn trust—among patients, providers, and caregivers alike.

The perspectives we share in this response reflect that vantage point. We welcome the opportunity to support CMS in building a digital ecosystem that works for everyone.

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PC-1

What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

Effective health management starts with medically trained providers and the necessary infrastructure to identify health conditions and establish a care pathway. However, clinical success ultimately depends on patient and caregiver compliance with care instructions. As the complexity of care increases — often due to multiple chronic conditions — adherence tends to decrease. The more complex the instructions, the lower the likelihood of successful outcomes.

To address this challenge, patients and caregivers need digital tools that consolidate information from today's fragmented, reactive, and unintuitive health landscape. Despite the proliferation of digital health products, few are meaningfully integrated into patients' daily routines or into clinical workflows. Aegis believes that the greatest value will come from digital software and services that are personalized, proactive, and seamlessly connected across care settings. Solutions must simplify — not add to — the cognitive burden on patients and caregivers. This is essential for promoting adherence and improving outcomes.

It is unlikely that a single solution will meet the needs of all patients. We recommend focusing first on populations managing multiple chronic conditions, where the complexity of care is highest and the potential for improving outcomes is greatest.

In parallel, Aegis is closely tracking legislative developments in the 118th and 119th Congress (specifically Energy and Commerce, Ways and Means, and HELP committees) regarding data security and patient data rights. We urge that any legislation on data sovereignty, data application, or data rights explicitly allow for permissible use cases that enable patients and caregivers to improve care delivery, without compromising data privacy or security.

a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

It is essential to assess healthcare quality through the perspectives of patients, clinical teams, and the family members who support them. From the patient's point of view, today's healthcare experience is often fragmented. Interoperability gaps arise the moment a patient moves beyond a health system's EHR. While Congress continues to strengthen patient data protection laws, these efforts can sometimes unintentionally restrict data sharing, discouraging the creation of the comprehensive, patient-controlled health information tools that patients and families want.

Digital health products can bridge these gaps. They have shown the ability to capture both objective data, such as vital signs, medication adherence, and sleep patterns, and subjective data, such as pain levels, mood, and overall well-being. Policies supporting remote physiologic monitoring have already demonstrated value in enabling this type of data collection and communication. Even the most widely used EMR digital front doors help but still leave significant functionality gaps.

We encourage CMS and Congress to ensure that data protection policies explicitly preserve patients' rights to control and consolidate their data in third-party tools of their choosing. Reimbursement models should also support tools and services that simplify care and empower patients and caregivers.

Patients managing multiple chronic conditions — such as diabetes, hypertension, and arthritis — should not have to rely on separate tools for each condition. Ideal digital products should consolidate data from EHRs and personal devices into one view and help users spot trends and take proactive steps before problems arise.

Accessible, actionable guidance is equally important. Many Medicare beneficiaries face challenges with health literacy. It is not enough to provide lists of articles or raw test results. Digital tools should offer clear, timely guidance in plain language (for example, "It's time for your eye exam" or "Your cholesterol is higher than last time; here's what that means.") The more intuitive the guidance, the more likely patients are to engage with and follow it.

Finally, scheduling care is another area where patients need better support. Many struggle to book, reschedule, or follow up with providers, especially when navigating multiple specialists. Tools that streamline appointment management and referrals across different providers would reduce friction and support better adherence to care plans.

b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

A digital personal health assistant could dramatically reduce friction and improve outcomes. Key tasks might include (but are not limited to):

- "Tell me what to do next."
  - Proactively surface the next recommended action based on a patient's condition, medications, or care plan. For example: "You haven't had a flu shot this year. Would you like help scheduling one?"
- "Coordinate my care."
  - Automate follow-up scheduling.
  - Track referrals and ensure communication between providers.
  - Provide visit preparation tools and post-visit summaries.
- "Explain what this means."
  - Translate clinical documents and lab results into plain English.
  - Help users understand side effects, warning signs, or medication instructions.
- "Keep my family in the loop."
  - Share key health updates and upcoming tasks with authorized caregivers.
  - Create shared task lists for caregiving responsibilities.
- "Find the right doctor or service."
  - Match patients with providers based on preferences, language, or specialty.
  - Show appointment availability and reviews.
- "Remind me and help me stay on track."
  - Send smart reminders tied to behavior, such as missing a medication log.
  - Encourage preventive actions like exercise or hydration.

PC-5

What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

CMS plays an important role in innovation adoption through several channels which includes administering Medicare and Medicaid, overseeing health and safety standards, and promoting innovation and quality. For Medicare beneficiaries and caregivers to adopt digital health products at scale, CMS must foster an environment of trust, accessibility, and ease of use. Many beneficiaries and their families are overwhelmed by complex care systems and unsure which tools are safe, effective, and worth their time. CMS can play a critical role in addressing these concerns by making digital health products more visible, easier to navigate, and better aligned with the real-world needs of patients and caregivers.

As the largest health payer in the U.S., CMS is uniquely positioned to open pathways for reimbursement for products that improve care quality, access, timeliness, and cost-effectiveness compared to the status quo. Aegis recommends that CMS update its approach to coverage, coding, and reimbursement for digital health products — particularly those that support screening, care coordination, and patient engagement.

Patient engagement tools often incur costs for providers or patients but can add meaningful value to patient care by enhancing adherence, understanding, and coordination. Without a reimbursement pathway, there is currently a fiscal disincentive to adopt these tools, even when they improve outcomes. We encourage CMS to explicitly include patient engagement and navigation tools in its reimbursement frameworks, so that providers and patients are not penalized for adopting technology that strengthens care.

Finally, we encourage CMS to promote the adoption of technologies that serve multiple functions and to update its reimbursement frameworks accordingly. One example is AI-enabled screening tools, which can initially be used to detect specific disease states but may evolve to cover a broader range of conditions as biomarkers and clinical evidence advance. To support timely adoption of such technologies, CMS should enable cross-walking of CPT and ICD-10 codes and proactively review and

update payment models. This may include broadening DRGs to reflect expanded screening capabilities and eligible populations. Without these updates, payment structures risk lagging behind the pace of technological innovation, limiting the potential benefits for patients and the healthcare system.

What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

We applaud the federal agencies for exploring how improvements in data access, reimbursement, and patient communication can promote better health outcomes.

Reimbursement is a key factor. There is a direct correlation between product adoption and reimbursement. However, the AMA coding process and CMS pathways for new technology reimbursement often lag behind the pace of innovation, resulting in access gaps. “Most favored nation” pricing laws can further constrain flexibility in coverage. Without a reimbursement pathway, patients, providers, or both bear the financial burden, which significantly flattens the adoption curve. This dynamic also exacerbates disparities, disproportionately affecting populations already at risk.

Data policies must also evolve in parallel. Many emerging tools, including those powered by AI and machine learning, require robust data inputs to generate clinically meaningful outputs. It is critical to ensure that patient data is protected and used appropriately. However, overly restrictive data policies may unintentionally prevent data use that can improve care models and outcomes. We urge policymakers to balance patient protection with thoughtful permissions that enable beneficial data sharing for care improvement.

Advances in digital tools are particularly important in a world where many chronic conditions are preventable and where sustained patient engagement can meaningfully impact outcomes. Increasing adoption of well-designed tools for screening, communication, and care efficiency is therefore a key priority.

Beyond technical and regulatory barriers, personal and social factors heavily influence technology adoption. A recent NIH study highlights that older adults and fiscally disadvantaged populations face specific barriers. Adoption is more likely when tools promote trust and support independent living. Trust, in particular, is a prerequisite for care adherence, yet it is subjective and varies across individuals. Many patients will not adopt a health app unless someone they trust recommends it and it fits easily into their daily routine. Trust can be quickly eroded when companies exploit data-sharing loopholes, as many consumers have experienced with app tracking and location monitoring by large platforms.

To promote broad adoption among Medicare populations, CMS should prioritize trust, simplicity, and relevance. One way to build trust is by placing digital tools where patients already engage with the system. If Medicare beneficiaries encounter approved apps listed on Medicare.gov, inside Medicare Advantage portals, or in Welcome to Medicare materials, they will be more likely to consider using them. Clinicians’ offices can also serve as important points of awareness for vetted tools.

Providers play a critical role in encouraging adoption. Patients often look to their doctors and care teams before trying new technologies. CMS can support providers by developing simple templates, talking points, and clinical guidance for recommending digital tools. If this falls outside of the purview of CMS, then alignment with societies would be a meaningful alternate step. Aligning these efforts with existing quality measures and care management programs would further ease provider adoption. It is also important to address coverage gaps. Providers are unlikely to introduce tools if doing so adds unreimbursed costs to care. We applaud Congress and CMS for creating codes for digital health services, such as Remote Physiologic Monitoring (RPM), which help mitigate this barrier.

Initial adoption of new tools should attempt to be frictionless. Many individuals abandon the process at the first hurdle—creating an account or verifying their identity. CMS should support the use of shared digital identity credentials to reduce friction and prevent patients from needing to manage multiple logins. A seamless onboarding experience matters more than most advanced product features.

Finally, digital tools must reflect the realities of the populations they serve. We encourage CMS to back grants or programs that explicitly require patient and caregiver input during the design process, particularly for underserved groups and individuals with limited digital literacy or English proficiency. The best way to build tools people will use is to involve them from the start.

PR-2

What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Several structural barriers limit the development and effective use of innovative applications in physician workflows. Addressing these will require policy, regulatory, and programmatic changes across CMS and HHS.

First, digital tools that support screening and prevention must be prioritized. Much of today's clinical workflow infrastructure is geared toward documentation and billing for existing care, not proactive prevention. Tools that enable earlier identification of disease risk and close screening gaps will have outsized impact on both quality and cost of care.

Second, regulatory timelines must better match the pace of scientific and technical innovation. While maintaining its strong commitment to safety, the FDA — in coordination with CMS — should accelerate pathways for adopting workflow innovations that support quality measurement, documentation, and billing. Without this, the U.S. risks falling behind global peers in bringing useful clinical technologies to scale.

Third, we must focus on workflow and data consolidation. Fragmented tools increase clinician burden and reduce adoption. Integrating quality reporting, clinical documentation, and billing into seamless workflows is critical for usability. The simpler the experience, the greater the likelihood of physician engagement and compliance.

Fourth, CMS and HHS should support a structured innovation pipeline that brings visibility to promising new technologies while avoiding undue commercial influence. Agencies need mechanisms to plan for, evaluate, and test emerging solutions — aligned to clear care improvement goals. The Real World Evidence (RWE)-driven efforts initiated under OASH did an excellent job of framing how the government should view areas of need in innovation. That project, run under Admiral Giroir, should be revitalized and expanded to inform this process. We should engage in public-private partnerships to showcase technologies to federal agencies. Federal agencies should play a role in areas of innovation need and align incentives to accomplish shared population health goals.

Finally, CMS should advance quality metrics that reward outcomes, not simply time spent or volume of services. Moving toward payment for success — measured by prevention, clinical improvement, and patient-centered outcomes — will create stronger incentives for physicians to adopt innovative workflow tools that genuinely improve care.

Taken together, these actions can help foster an ecosystem where technology innovation translates into practical, measurable gains in physician workflows and patient outcomes.

TD-1

What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Many developers want to build for Medicare populations but are discouraged by structural challenges. The problem isn't a lack of ideas—it's that the path to building something useful, trusted, and scalable is unclear. Developers face uncertainty around data access, long product cycles, and limited visibility into what CMS values in a digital tool. This slows innovation and prevents patients and caregivers from benefiting from tools that already exist in other sectors.

We defer to Congress and the relevant legislative bodies on the most appropriate pathways for enacting the changes needed to support this innovation. However, within CMS's role, we see several practical steps that can accelerate progress.

In the short term, CMS can expand access to relevant data through developer-friendly infrastructure. While claims data is a starting point, developers also need information about care plans, referrals, functional status, and social risk factors. Many of these data types remain locked in siloed systems or are only accessible through slow, manual processes. CMS should provide more sample datasets, clearer onboarding documentation, and sandbox environments populated with realistic, de-identified or synthetic data. These sandboxes would enable the public and the development community to prototype, test, and refine digital tools without the delays of securing live data integration.

Second, CMS should consider launching a voluntary recognition program for digital health tools. Many patients and caregivers hesitate to trust new apps unless they are recommended by a provider or endorsed by a credible source. A CMS-led recognition framework — grounded in usability, safety, and accessibility — would help developers align to important patient-centered outcomes and offer a clear signal of trustworthiness to the market. Such a structure would lower market risk and increase developer interest.

Third, CMS can sponsor targeted pilot programs focused on priority use cases, such as care navigation for patients managing multiple chronic conditions or tools that support caregiver coordination across providers. These pilots need not be large but should feature clear goals, lightweight evaluation processes, and a visible pathway toward broader scaling or recognition. Developers are far more likely to invest when there is a structured way to test solutions in a Medicare-relevant environment without taking on prohibitive costs or risk.

In the longer term, CMS should focus on building the technical foundation to support consumer-facing integration with health and administrative systems. Most current APIs are optimized for enterprise uses, not patient-driven experiences. CMS could collaborate with ONC and industry stakeholders to standardize API access for core functions like scheduling, benefits verification, and identity management—capabilities that are essential for building the tools patients actually want to use but are often underserved by current interoperability efforts.

Finally, CMS can further align digital health innovation with value-based care. Tools that help patients adhere to care plans, avoid unnecessary utilization, or manage conditions at home drive value—but only if care models and payment programs incentivize their use. By explicitly recognizing and rewarding the role of digital tools in these programs, CMS can signal demand to the developer community and help these innovations reach the populations that stand to benefit most.

Developers do not expect CMS to remove all friction, but they need clarity, accessible infrastructure, and visible pathways to scale. The steps outlined above, both near-term and structural, can help shift the environment from uncertain to possible and unlock a wave of patient-centered innovation for the Medicare population.

Closing and Signatories

At Aegis Ventures and through the Digital Consortium, we have seen firsthand both the opportunities and the persistent barriers to adopting digital tools that improve care for Medicare beneficiaries. We know that technology alone is not enough—what matters is building solutions that fit the realities of patient, caregiver, and provider experiences.

The next phase of progress will require deep collaboration between government, providers, and technology developers. We believe CMS is uniquely positioned to help shape a digital ecosystem that promotes trust, transparency, and better outcomes—while ensuring that innovation serves all populations, including those historically underserved.

We are committed to partnering with CMS in this effort. Our work across diverse health systems has shown that when tools are co-developed with those on the front lines of care, they are more likely to earn trust, drive adoption, and deliver measurable value.

We appreciate the opportunity to contribute to this dialogue and stand ready to support CMS in building a digital health future that works for every patient.

This response is jointly submitted by Aegis Ventures on behalf of the Digital Consortium, the Aegis portfolio companies, and Dr. John Noseworthy.